National Cancer Control Programme (NCCP) 00738 · clinician draft

Bevacizumab + weekly paclitaxel

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

First paclitaxel dose · combination Day 1

Active medicines: Bevacizumab + Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before each administered chemotherapy dose Review QT risk, constipation, liver function and any steroid already prescribed.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be confirmed Review contraindications, renal function and interactions.

Additional phase-specific support

- Before the first paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment.
- The first and subsequent infusion-reaction plans are different; do not automatically repeat the initial steroid schedule.
- Check blood pressure (BP) before each bevacizumab dose and reconcile urine-protein results, bleeding, thrombosis and wound healing.
- National Cancer Control Programme (NCCP): withhold bevacizumab 28 days before major or 7 days before minor elective surgery. Do not restart for at least 28 days after major surgery and until the wound is fully healed; the team individualises this.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00738 v3 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.19 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Day 8 · paclitaxel only

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before each administered chemotherapy dose Review QT risk, constipation, liver function and any steroid already prescribed.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be confirmed Review contraindications, renal function and interactions.

Additional phase-specific support

- From Day 8 onward, no routine dexamethasone is listed in the suggested table; chlorphenamine 10 mg intravenous (IV) 30 minutes before treatment continues.
- Famotidine 20 mg IV 30 minutes before may be omitted after tolerated treatment under consultant/local guidance. A previous reaction requires an individual rechallenge plan.
- Bevacizumab is not scheduled on Day 8; continue to report its adverse effects.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00738 v3 · p6 · NCCP antiemetics v8, Table 1 pp4–5

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Later Days 1/15 · combination after tolerated paclitaxel

Active medicines: Bevacizumab + Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Before each administered chemotherapy dose Review QT risk, constipation, liver function and any steroid already prescribed.
Metoclopramide	10 mg orally up to three times daily as needed	Rescue duration to be confirmed Review contraindications, renal function and interactions.

Additional phase-specific support

- From Day 8 onward, no routine dexamethasone is listed in the suggested table; chlorphenamine 10 mg intravenous (IV) 30 minutes before treatment continues.
- Famotidine 20 mg IV 30 minutes before may be omitted after tolerated treatment under consultant/local guidance. A previous reaction requires an individual rechallenge plan.
- Check blood pressure (BP) before each bevacizumab dose and reconcile urine-protein results, bleeding, thrombosis and wound healing.
- National Cancer Control Programme (NCCP): withhold bevacizumab 28 days before major or 7 days before minor elective surgery. Do not restart for at least 28 days after major surgery and until the wound is fully healed; the team individualises this.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00738 v3 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-25 · SACTCheck v0.84.19 · Regimen-level drafts; prescriber and oncology-pharmacy verification required